



DEPARTMENT OF HEALTH AND HUMAN SERVICES

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Food and Drug Administration
Silver Spring, MD 20993

**CENTER FOR BIOLOGICS EVALUATION AND RESEARCH
OFFICE OF VACCINES RESEARCH AND REVIEW**

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DATE: December 2, 2020 **PAGES:** 2

TO: **BioNTech RNA Pharmaceuticals GmbH/Pfizer, Inc.**
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FROM: (b) (6)
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CBER Reference: EUA 27034.0

EUA Title: Human Coronavirus mRNA Vaccine (SARS-CoV-2 Spike Protein; BNT162b2) in Lipid Nanoparticles (ALC-0315, ALC-0159, DSPC and Cholesterol) (Pfizer-BioNTech COVID-19 Vaccine)

SUBJECT: CBER comments regarding cases of Bell's palsy and the active surveillance studies (Study Protocols C4591008, C4591011 and C4591012)

Dear Ms. Harkins:

Reference is made to your initial EUA 27034 submission dated November 20, 2020. We have the following comments regarding cases of Bell's palsy and the active surveillance study (Study Protocols C4591008, C4591011 and C4591012).

1. Bell's palsy. You state on page 146 of the EUA request document that two subjects in the BNT162b2 group reported facial paralysis (Bell's palsy). In both cases, symptoms appeared temporally associated with vaccination. Please address the following requests regarding cases of Bell's palsy:
 - a. Please provide the complete case narratives for the two cases of Bell's palsy described in the EUA, and the case narratives for any other cases of facial paralysis that occurred.
 - b. Please add Bell's palsy as an adverse event of special interest (AESI) to your three active surveillance study protocols submitted to EUA 27034 (Protocol numbers C4591008, C4591011 and C4591012).
2. Active surveillance studies. Please address the following concerns regarding your active surveillance studies:
 - a. Healthcare Workers Cohort Protocol (C4591008)
 - You submitted a protocol for an active surveillance study in healthcare workers (Study Protocol C4591008). You include Multisystem Inflammatory Syndrome (MIS) in Children as an AESI. However, MIS can occur in adults as well. Therefore, please consider MIS in any age group as an AESI, not just in children.
 - It appears that the study entirely relies on self-enrollment, self-data collection, and self-reporting by participants. We are concerned that this study process poses several challenges including:
 - Not recruiting sufficient participants,
 - The potential for a large proportion of missing or inaccurate data, and
 - The potential for a large proportion of the study population being lost to follow-up.

These challenges will have major impact on the accuracy, reliability, and generalizability of the study results. Please consider modifications to the study design in terms of recruitment, enrollment, data collection, and follow-up strategies so that the study burden will not fall heavily on the participants.

- b. Veterans' Affairs (VA) Active Surveillance Protocol (C4591012) and the Department of Defense (DoD) Safety Surveillance Protocol (C4591011): Please confirm whether Pfizer has already secured agreements with VA and DoD to have access to their data and conduct the proposed studies, since our previous experience shows that VA and DoD rarely provide their data even to other government agencies. If you do not have an agreement in place, please describe how you plan to conduct these studies.
3. Response to information request. You provided your response to the Information Request (IR) regarding the Pharmacovigilance Plan in EUA 27034.6. Please provide the updated PVP data capture aid documents that incorporate the changes to which you agreed in your response to the IR. This document should be provided to FDA for review by December 7, 2020. Please ensure that the only changes are exactly the same as was indicated in amendment 6 and indicate this in the cover letter of the submission.

If you have any questions, please contact me by email: (b) (6) [@fda.hhs.gov](mailto:(b) (6)@fda.hhs.gov) or at (b) (6).